



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض ربيعاً : سوزان انور مصطفى السيد

المحافظة : الجيزة

الحي / المدينة : بولاق الدكرور

العنوان : 15 ش عبدالفتاح - ش

النزهة-بولاق الدكرور-الجيزة

تاريخ تسجيل الإستمارة : 27-09-2026

رقم البطاقة : 27503032101821 رقم التليفون : 01066659144

العمر(تاريخ الميلاد) : 03-03-1975 المهنة : ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإحصائي الاجتماعي (.....) التوقيع: (.....)

المقر بما فيه التوقيع: (.....)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : أبحاث أورام مزيد من الابحاث و الاشعات [7954] قسم العلاج : الاورام

التعليق على التشخيص :

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني

الاسم : د/ندى حسن

الاسم : د/نرمين رمضان

الوظيفة :

الوظيفة :

الوظيفة :

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة

رئيس اللجنة ومدير المستشفى.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

أ.د. / /د. / /د. / /د. /

قرار اللجنة

التشخيص النهائي :

خطة العلاج :

المستشفى :

قسم :

جهة العلاج :

نفقات :

ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

--

Name: سوزان انور مصطفى السيد National ID: 27503032101821	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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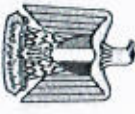
وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حميد بالثدى و تحتاج الي Further investigations and imaging

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

DM5
asf

٢٠٢٥/٠٤ ٢٧٥.٣.٣٢١.١٨٢١
رَبِّهِ مَنَزَل
مَنْشِي
هَنْطِي مَحْطَقِي جَوهر السَّيِّد
الْبَطَاقَةُ سَارِيَّة حَتَّى ٢٠٣٢/٠٤/٢٨ مَرْجُوَّة



سوزان
أنور محطقي السيد
١٦ ش. طابو التاجية
القي - الجزيرة

جَمَاهِيرِيَّةُ كُرْدِسْتَانِ
بَطَاقَةُ تَحْقِيقِ الشَّخْصِيَّةِ



٢٧٥.٣.٣٢١.١٨٢١
LC6224317

03/08/2026

MDT FORM

Name:	Souzan Anwar Moustafa /	MRN:	1260
Age:	51 Y	ID:	٤٧٥٢-٢٢١-١٨٤١
Menopausal Sta	post	Phone:	01066659144
NRS SCORE:		Marital Status:	
BMI:		Family History:	
CO Morbidity:	hx of RT breast cancer >> mastectomy + implant	Reason Of MDT:	

Clinical Data:

BX:

ER: 8/8 PR: 2/8 HER2: +2 not ar KI67: 25%

Sonomammography:

Mets Workup:

ct cap >> tiny pul nodule + S1 mixed lytic / sclerotic osseous lesion

History:

It breast cancer was planned for NACT if bone scan -ve but bone scan showing active mets osseous lesion at LV3
for reMDT

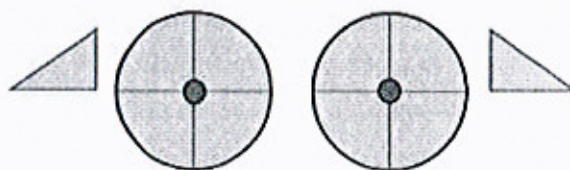
Recommendations:

for femara with zometa every 3 ms

MDT Outcome & Plan

R

L



T N M

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
د. حنان عزت	د. احمد نجيب				

Name: سوزان انور مصطفى		
MR. : 1260		
Detailed Medical Report		
		السيد
تخية طيبة وبعد,,		
	بشأن تقرير طبي للمريض	إيماء للطلب المقدم من
	رقم طبي:	بقسم:
ننشرف بأن نوضح فيما يلي تقريراً عن حالته		
	تاريخ وساعة الخروج :	تاريخ وساعة الدخول :
وبعد الكشف والفحوصات والمتابعة وجد انه		
The patient underwent a bone scan that demonstrated active metastatic disease involving the L3 vertebral body, and correlation with other radiological findings was requested. The patient has a history of invasive ductal carcinoma, grade II, hormone receptor–positive and HER2-negative. She received right-sided modified radical mastectomy in June 2019 for a pT3N1M0 tumor, followed by adjuvant chemotherapy consisting of three cycles of FEC and three cycles of docetaxel, then postoperative radiotherapy and five years of tamoxifen. In February 2026, a new left-breast mass was identified and biopsied as invasive ductal carcinoma with estrogen receptor 8/8, progesterone receptor 2/8, and HER2 2+. Breast MRI revealed multicentric lesions in the left breast at the 2- and 10-o'clock positions with pathological lymph nodes (T1, N2). A bone scan showed metastasis at L3. The multidisciplinary team discussed initiation of letrozole (Femara) and zoledronic acid (Zometa); the patient reported self-funded use of these agents for two months while awaiting formal approval, and a new approval request has been submitted. A CT of the chest, abdomen, and pelvis was performed, and the patient was cleared to continue letrozole while awaiting a repeat bone scan to assess metastatic breast cancer involvement of bone. The current plan is to maintain endocrine therapy with letrozole and monitor disease status with imaging.		
تحريرا في ,		
	التاريخ : 19-09-2026	الوقت :
	نرمين مصطفى عبد القادر بهي الدين	الطبيب المعالج :
يعتمد ,		

Name: Mrs. Suzan Anwar Mustafa Al-sayed
Date: 2025-11-27

Bilateral Digital soft tissue mammography with high resolution breast sonography revealed:
Clinical summary: Post right MRM & ALAND with silicone implant, complaining of left axillary lump of 1 month duration.

Findings:

Right breast.

- Almost entirely fatty breast parenchyma. (ACR a)
- Intact silicone implant with no sign of intra or extra-capsular rupture.
- Thickened skin.
- Ultrasound demonstrated intact silicone implant with no sign of intra or extra-capsular rupture
- No solid or cystic lesions or fluid collections.
- Thickened skin reaching 3.5 mm.
- Clear right axilla. No infra or supraclavicular lymph nodes.

Left breast.

- Heterogeneously dense breast parenchyma that may obscure small lesions. (ACR c)
- Multiple irregular speculated high density mass lesions with surrounding distortion:
- Ultrasound demonstrated Multiple irregular spiculated heterogenous hypoechoic solid mass lesions with surrounding echogenic halo, posterior acoustic shadowing and mild internal vascularity on color Doppler study largest are seen at:
 - Index Lesion: 2 o'clock, 6 cm from nipple, measuring 13 x 16 mm.
 - 12 o'clock, measuring 10 x 5 mm.
 - 10 o'clock, 9 x 8 mm.
 - Multiple satellites are seen at the rest of the breast parenchyma measuring 4-7 mm each.
- 4 o'clock, non-specific intra-mammary lymph node measuring 4.4 x 6.5 mm with 2 mm cortical thickness.
- Normal skin/nipple/areola complex.
- Multiple level 1 and 2 pathological axillary LNs (more than three). No supraclavicular pathological axillary nodes.

Conclusion:-

Right breast (BIRADS 2): clear mastectomy bed and axilla with intact silicone implant.

Left breast (BIRADS 5): Multicentric pathological process with ipsilateral pathological axillary lymph nodes (level 1 & 2).

Recommendation:-

For CESM & biopsy and histopathological correlation.

Best Regards,

DR. Amal Hegazy, M.Sc.

BIRADS classification of breast lesions according to the American College of Radiology used for breast assessment combined with

Category & recommendations

recommendations:

Suspected Cancer Risk

BIRADS 0: Non-informative study.	
BIRADS 1: Normal study, routine screening	0 %
BIRADS 2: Benign findings, routine screening	0 %
BIRADS 3: Probably benign, for close follow up.	2 %
BIRADS 4: Probably malignant, for biopsy.	50 %
	A: mild suspicion.
	B: moderate suspicion.
	C: high suspicion.
BIRADS 5: Malignant, for biopsy.	98-100%
BIRADS 6: Pathologically proven malignant	



Patient Name: Suzan Anwar Moustafa Al Sayed	Date : 14/10/2024
Exam : MAMMOGRAPHY BOTH	Age : 49 Year(s)
Medical ID: 1691/2019	Pat. ID: 11526

Left BREAST MAMMOGRAPHY WITH BILATERAL US

Indication:

Patient had right MRM with implant reconstruction and left breast reduction mammoplasty

Technique

- Left breast MLO and CC views
- Bilateral ultrasound

Report:

- scattered fibroglandular densities of the left breast (ACR B)
- No spiculated masses or grouped microcalcifications.
- Complementary ultrasound also showed no left breast solid or cystic masses
- Normal skin thickness and nipple-areola complex, with average caliber of the retro-areolar ducts
- Nonspecific left axillary LN, with preserved fatty hila and uniform cortical thickness.
- Free right operative bed with no evidence of recurrent mass lesions with intact implant
- Free right infra and supra clavicular regions.

Opinion:

- Right MRM with free operative bed with no evidence of recurrent mass lesions
- No left breast suspicious findings (BIRADS 2)

Yours truly

Signature Dr. Yasmin Nada

14/10/2024 12:16 PM



Pathology Report

Name: سوزان انور

Age: 43 years.

Clinical data: Right breast cancer.

Specimen type: Right MRM

Date: 22/6/2019

Lab. Number: 450

Hospital Number: 1691/2019

Gross Description:

Mastectomy specimen; 23x17x9 cm partly covered by skin ellipse; 22x16 cm with attached axilla 10x10 cm.

Sectioning revealed a retro-areolar ill-defined widely extending firm greyish white mass; 11x10x2 cm, located 1cm deep to areolar skin, with least deep margin 6cm. the mass was enclosing nodular area; 1x1 cm.

Axillary dissection revealed 18 nodules up to 1x1 cm.

Microscopic Examination:

Histopathological examination of sections prepared from the mass revealed invasive cores and tubules of moderately anaplastic ductal epithelial cells associated with 10% DCIS. Positive lymphatic tumor emboli and fair desmoplastic stroma.

The nipple, areola & resection margins were free of tumor tissue.

There were positive metastatic deposits in 1/18 axillary lymph nodes with focal extra-capsular extension.

Diagnosis:

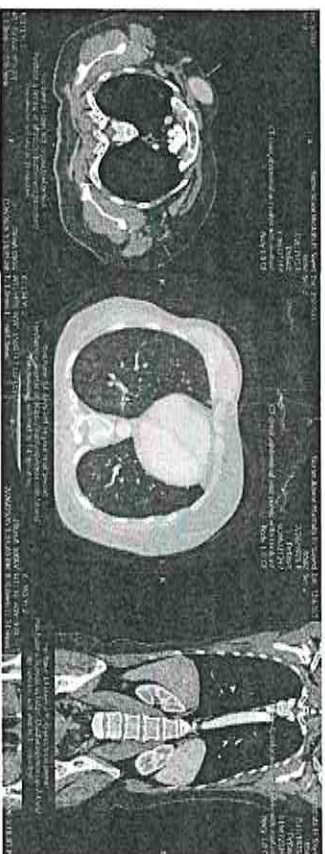
- Right MRM,
- Invasive duct carcinoma GII
- 10% DCIS
- Positive lymphatic tumor emboli.
- No perineural invasion.
- Free nipple, areola & resection margins.
- Positive 1/18 axillary lymph nodes with extra-capsular extension.
- Stage PT3 PN1.
- For IHC; (ER, PR, Her2 & Ki67)

Prof. Dr. Amal Haridy

Prof. Dr. Essam Ayad

Patient's Name:
Date:
Referred by:
Hospital:

Suzan Anwar Mustafa El-Sayed
2026-07-26
Prof.Dr. no doctor
Goustave Roussey International Egypt



MULTISLICE CT EXAMINATION OF THE CHEST ,ABDOMEN AND PELVIS WITH CONTRAST ADMINISTRATION REVEALED:-

- Evident right mammoplasty with intact silicon stent seen inside and no evidences of either residual/recurrent or newly-developed neoplastic lesions seen in the breasts.
- Normal right axilla.
- **Benign -looking left axillary lymph nodes are seen with preserved their hilar fat contents and the largest one measures about 11mm.**
- Hyperinflated both lungs.
- No sizable abnormally-enhancing pulmonary nodules ,patchy areas of consolidations or foci of calcifications noted.
- No sizable abnormally-enhancing mediastinal or hilar lymph nodes seen.
- Patent central airways.
- No pleural or pericardial effusion.
- Average size of the heart with normal CT of its big vessels.
- **Normal bones of the thoracic cage apart from spondylo-degenerative changes of the scanned spine.**
- Average sized liver,measuring about 14.4 cm, showing a homogenous parenchymal attenuation and enhancement pattern.
- No sizable abnormally-enhancing focal lesions or dilated biliary radicles.
- Patent PV.
- Average sized spleen,measuring about 10.7 cm. No focal lesions seen inside.
- Normal CT appearance of the pancreas, functioning both kidneys, both suprarenal glands, aorta and IVC.
- No sizable abdominal or pelvic lymphadenopathy.
- No ascites.
- No intra-vesical stones,solid masses,wall thickening or diverticulosis seen.
- The rest of the pelvic structures are unremarkable.

Name : Suzan Anwar Mustafa

Date : 10-09-2026 03:53

Branch : Dokki

Medical No. : 459030

Ref.Physician Prof : Goustav Roussey Egypt

تعليق مستشفى : Ins.Company

NUCLEAR MEDICINE BONE SCAN REPORT

52-year-old female with breast carcinoma, status post left mastectomy, chemotherapy, and radiotherapy (2019), currently on hormonal therapy. Known bone involvement at L3, presenting with generalized bony aches.

Procedure: Whole-body planar bone scintigraphy, anterior and posterior views, following intravenous administration of 99mTc-MDP.

Findings:

- Thoracolumbar scoliosis with rightward convexity is noted.
- A focal area of intense increased radiotracer uptake is identified at the L3 vertebral body, corresponding to the reported lesion.
- The remainder of the axial and appendicular skeleton shows symmetrical physiological tracer distribution without additional focal lesions.
- Renal excretion is prompt and symmetric. No abnormal soft tissue uptake.

Impression:

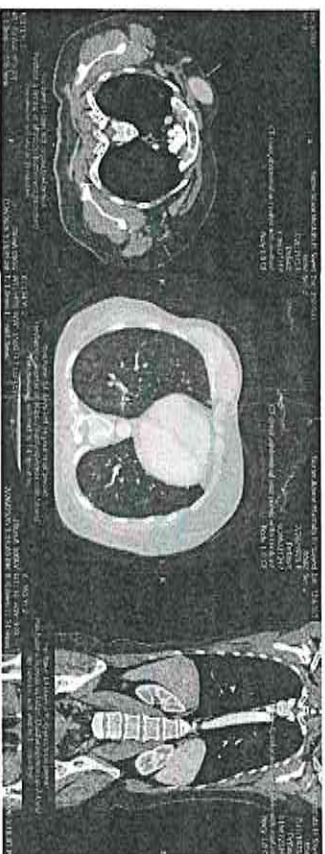
- Solitary focal osteoblastic lesion at L3, suspicious for secondary osseous metastatic involvement in the clinical context.
- No evidence of additional active osteoblastic metastases in axial or appendicular skeleton.
- Clinical and radiological follow-up as indicated.

Dr. Alaa Nagi

MD Nuclear Medicine

Patient's Name:
Date:
Referred by:
Hospital:

Suzan Anwar Mustafa El-Sayed
2026-07-26
Prof.Dr. no doctor
Goustave Roussey International Egypt



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Ref.Physician Prof : Goustav Roussey Egypt

تعاقب مستشفيات : Ins.Company

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